

Like-minded, like-bodied: How users (18-26) trust online eating and health information

Rachel Xu
Jigsaw, United States
rachelxu@google.com

Rebekah Park
Gemic, United States
rebekah.park@gemic.com

Nhu Le
Gemic, United States
nhu.le@gemic.com

Laura Murray
Gemic, United States
laura.murray@gemic.com

ABSTRACT

This paper investigates the relationship between social media and eating practices amongst 42 internet users aged 18-26. We conducted in-person ethnographic research in the US and India to observe how young people navigated eating and health information online. Participants portrayed themselves online using a vocabulary we have labeled “the good life”: performing holistic health through diet, exercise, and idealized lifestyles. We observed participants engaging in behaviors of disordered eating while actively eschewing them. When online, participants valued personal testimonies, and readily tested tips from content creators who they believed shared similar beliefs and bodies to them. In doing so, they did not engage in probabilistic thinking and opened themselves to potential harm. We found that social media feeds did not unidirectionally influence participants—they often reflected participants’ internalized views of health, in an intertwined, non-linear journey. Reducing the online spread of disordered eating practices requires addressing it within young people’s social context.

CCS CONCEPTS

• **Human-centered computing** → Human computer interaction (HCI); HCI design and evaluation methods; Field studies; • **Social and professional topics** → User characteristics; Cultural characteristics.

KEYWORDS

disordered eating, trust, information sensibility, eating and health

ACM Reference Format:

Rachel Xu, Nhu Le, Rebekah Park, and Laura Murray. 2024. Like-minded, like-bodied: How users (18-26) trust online eating and health information. In *Extended Abstracts of the CHI Conference on Human Factors in Computing Systems (CHI EA '24)*, May 11–16, 2024, Honolulu, HI, USA. ACM, New York, NY, USA, 7 pages. <https://doi.org/10.1145/3613905.3650932>

1 INTRODUCTION

During the COVID-19 pandemic, reports of eating disorders among youth skyrocketed [1]. Scholars inferred that increased social media

usage—which had also increased due to lockdowns, loss of routines, and other hardships—led to a spike in disordered eating behavior [2]. To examine the relationship between their social media feeds and their eating and health practices, we set out in 2022 to investigate how youth navigate eating and health information online in the US and India.

We conducted ethnographic research with 42 internet users aged 18-26 to observe how they navigated online content related to eating and health. We focused on self-reported experiences with disordered eating habits, and did not require formal diagnoses, to understand eating and health cultures beyond the pathology of diagnoses. Our research questions included: (RQ1) How do broader eating-related health cultures shape information journeys? (RQ2) What pathways or patterns do young people experience when searching for information about eating and health? (RQ3) What information do they trust and why?

Our analysis yielded several key findings:

- Individual pieces of content could not be universally classified as helpful or harmful. The same piece of content (for example, content intended to showcase “eating recovery”) could help one person but trigger another, or elicit different responses at different times from the same person.
- Participants used a coded vocabulary of images and words for disordered eating: the pursuit of “the good life.” Participants sought to perform the good life by signaling holistic health and wellness across all areas of life (work, relationships, hobbies, diet, and fitness), while maintaining the illusion that they had not sacrificed anything to achieve it.
- The scientific basis for claims found online about eating and health rarely mattered for our participants. They assessed trustworthiness through whether a claim felt personally applicable, and whether the messengers were “like-minded” and “like-bodied” as a heuristic.
- Participants evaluated content through immediate self-experimentation, without conducting additional online searches about the scientific basis behind claims or engaging in probabilistic thinking.
- Participants with racially diverse backgrounds, along with queer and disabled participants, often found creators and communities online where they felt represented, in stark contrast to the spaces they inhabited offline or in mainstream media. Despite finding solace in online communities, many participants confessed to feeling that their lives would improve if they spent less time on their phones, and sought

Permission to make digital or hard copies of all or part of this work for personal or classroom use is granted without fee provided that copies are not made or distributed for profit or commercial advantage and that copies bear this notice and the full citation on the first page. Copyrights for third-party components of this work must be honored. For all other uses, contact the owner/author(s).
CHI EA '24, May 11–16, 2024, Honolulu, HI, USA
© 2024 Copyright held by the owner/author(s).
ACM ISBN 979-8-4007-0331-7/24/05
<https://doi.org/10.1145/3613905.3650932>

off-ramps (leveraging their online communities) into the “real world.”

These findings have important implications for the HCI community. Often, we think of explicit disordered eating content as distinct from “normal” content. However, our findings show that targeting explicitly disordered eating content alone is insufficient to support young adults. We argue in this paper that the eating and health content participants consumed in 2022 made conversations about disordered eating more accessible and palatable to mainstream users. The aspirational good life makes dieting behaviors more acceptable, by depicting the goal of such behaviors as not primarily to be thin, but to live life in “the right way” with a socially-ideal body. Free from the negative stigma associated with explicit disordered eating discourse, the good life enabled participants to self-experiment with knowledge they gained from like-minded and like-bodied communities without guilt, and without reflecting on potential harms that could result from their quick actions.

2 LITERATURE REVIEW

For the past 40 years, there has been broad agreement that etiological models of eating disorders “should be multidimensional” [3]. Clinicians, theorists, and researchers agree that there is no single cause or set of causes to which disordered eating behavior can be traced [4, 5]. Sociocultural influences such as media exposure, pressure for thinness, and thin-ideal internalization, for example, can exacerbate—or be exacerbated by—personal characteristics (negative emotionality, perfectionism, or negative urgency) [6].

Efforts to factor in the role of culture in biological models of eating disorders often reduce culture to a “risk factor.” Culture is most often used, in fact, as a reductive “proxy for racial or ethnic groups” [7]. According to anthropologist Rebecca Lester, research that approaches culture as a static variable is not entirely without value.¹ But in focusing on cultural membership as a risk factor and on individuals as passive recipients of media images, these approaches cannot account for how people use practices around eating and health—including disordered eating habits—to actively make meaning in their lives. Lester argues that disordered eating behaviors are not about pathological fear of food and eating, but rather about a yearning to be perceived as a good and valuable person in the world, expressed through the body as the fundamental unit of identity—which Lester described as the “deep why.” To get at these deeper questions linking behavior to belief, identity, and aspiration, we need to explore “how the practices of body discipline and affective modulation shape a person’s sense of being in the world” [8].

From sub-cultures of carnivory to warring factions of vegan YouTubers, there are a growing number of cultural identities accessible to the mainstream [9, 10]. A growing number of online cultures, subcultures, and countercultures exist to help people navigate the terrain related to health and eating [11, 12]. Currently, however, we lack a robust literature about the proliferation of health-related identities among young people to match the comparatively well-established body of research on their “networked” lives [13–16]. We sought to understand how new identities—and

the sense of community affiliation they make possible—were mediated by online information. We conducted this research during the COVID-19 pandemic, which saw a rise in reports of disordered eating behaviors among young people [1], that were linked to lockdowns, uncertainties, and increased social media usage. Other media theorists have noted that with the increased digitization of social life with the pandemic, teens and young adults have worked out new ways to care for each other through online spaces [17].

Academic debates on internet usage and eating disorders have frequently focused on ethical questions regarding “pro-anorexia” (“pro-ana”) websites and forums which portray extreme fasting as a lifestyle choice. Not all dietary misinformation, however, can be as easily identified. “Thinspo,” or content that seeks to motivate the user to work towards thinness, has been shown “to have a negative impact on viewers with regard to body image. . . even five minutes of exposure to thin and beautiful images” can negatively affect body image [18]. Basing their research on a survey of pro-ana internet searches and websites, Lewis and Arbuthnot argue that “search terms with references to thinspiration and thinspo are associated with the most harmful Website content” [19]. Some content may then be deemed harmful by its ability to send users into a “toxic spiral” where content becomes intensified and more explicitly harmful [20].

At the same time, however, there is still a limited understanding of how and when tech companies can successfully moderate a user’s informational health ecosystem. Trigger warnings and censorship, for example, have not historically worked [21, 22]. The challenge is made more complex by the differing discourses on disordered eating in diverse cultures. Of particular relevance to this study is India, which scholars like Krishnendu Ray have observed have no known language or reference points for eating disorders.² Moreover, critics have argued that dieting apps that use an educational model fail to understand that poor health decision-making does not come from lack of knowledge, but from social stratifications and differing definitions of health [23]. Such critiques highlight the gap between conventional paradigms of information literacy and what young people actually do online [24].

3 METHODOLOGY

We conducted a qualitative research study from June to October 2022 with 42 participants aged 18–26 (Appendix A1), who were engaged in eating and health online content. Half were from the US and half from India. We sought to understand eating and health cultures separate from the pathology of disordered eating. As such, we intentionally did not recruit participants with diagnoses of an eating disorder from a medical professional in the US, and were aware that eating disorders are not an established concept in India as they are in the US. Instead, we relied upon self-reported experiences with disordered eating and recovery in the US, and time and energy spent on eating and exercise in India.

3.1 Research ethics

Participants gave informed written consent during the recruitment process and confirmed their consent before in-person ethnography

¹Interview conducted on June 30, 2022 with Rebecca Lester.

²Interview conducted on July 14, 2022 with Krishnendu Ray.

commenced. A licensed psychologist specializing in eating disorders reviewed all prospective participants' answers to our screener, to confirm that we excluded participants whose responses indicated risk for distress. With this expert, we designed a de-escalation procedure in the event that a participant exhibited distress or disclosed sensitive medical details. We maintained strict data privacy protocols for all participants. All data collected used only pseudonyms assigned to each participant. All participants were instructed to withhold personally identifying information (PII). Raw interview and diary data was scrubbed of PII during transcription, and we have omitted personally identifying details or phrases from quotes in this paper. Pseudonyms used below are not the same pseudonyms used in data collection.

3.2 Limitations

Our study's sample was not statistically representative of the broader population. We limited the study to two geographic locations accessible to urban areas. However, we sought to diversify the sample because we were conscious of historical biases of research into disordered eating (focusing on participants who were predominantly white, wealthy, and female). We deliberately included male participants in both countries, and queer and non-white participants in the US. We also chose to diversify our sample across geographic density, income, and eating and health behaviors—however, there may have been other characteristics (e.g. disability)³ that may have been relevant to eating and health cultures.

3.3 Research Methods

Our study had three phases of data collection.

Phase 1: Literature review and diary study: Our literature review and expert interviews framed our understanding of how disordered eating has evolved in the age of technology, and how the two geographies differ culturally and historically when it comes to eating and health. Each participant created a collage of images that reflected their ideas of what is “healthy” and “unhealthy.” These collages were used as a starting point for conversations about their information sources and their personal assessments of their own health.

Phase 2: Participant observation and interviews: In two encounters (one in-person, one digital) over one month with each participant, researchers probed into: 1) life histories; 2) information ecosystems; and 3) experience with disordered eating (if applicable). We met with participants in their homes or in their neighborhoods, and immersed ourselves in their daily lives and online media consumption. This included joining them on eating- or health-related activities of their choosing, such as eating at their favorite neighborhood spot, grocery shopping and preparing meals in their kitchen, or jogging along their routine route in a local park near where they lived. In doing so, we observed the ways people's words and actions were sometimes at odds with each other, as they navigated the emotionally and morally fraught territory of eating and health.

Phase 3: Data synthesis and remote follow-up interviews: Following the in-person observation and interviews, we conducted thematic analysis of the data to identify an initial set of participant

needs and insights [25, 26]. We workshoped these with other researchers, product managers, and policy specialists from Google working on issues of health and user safety. We shared our discussions, including intervention ideas, in follow-up interviews with participants to collect more qualitative data and to validate our interpretations of their experiences (i.e. member checking).

3.4 Data Analysis

Preliminary analysis began immediately after the in-person ethnography based on field notes. The research team discussed the interview and diary study data after watching the recordings and reading participants' textual responses, and identified insights related to our RQs. This allowed us to tailor the follow-up remote interviews to each participant. During both rounds of interviews, we coded collected data iteratively, yielding 150+ discrete insights [27]. We used the coding to assess the saturation of insight across the dataset as a whole and among specific subsets of participants. After data collection and coding were complete, we iteratively clustered insights into themes, and created rough thematic maps. We did this across several collaborative and independent analysis sessions.

4 RESULTS & DISCUSSION

4.1 “The Good Life”

In our diary study, we asked participants to create two collages of images found on the internet that represented “healthy” and “unhealthy” to them personally. We learned that the same content could be healthy or unhealthy depending on the participant or where they were in their journey with eating and health. All participants internalized ideas about eating and health at home, from their families, and when their attitudes and behaviors changed over time, they felt that their social media feeds reflected, not caused, their internal responses to content around bodies, dieting, and exercise. For example, Jaime (23, US) explained that prior to adopting a “recovery mindset,” content intended to help people like themselves overcome anorexia had the opposite effect:

“[For me] this recovery stuff is more triggering and instructive... When I was 12, it helped me learn how people were engaging in [anorexic] behaviors.”

Today, Jaime can view and critique pro-ana content and not be triggered by it. Many participants shared Jaime's experience: content was not categorically wholesale healthy or unhealthy. Content unremarkable or helpful to one person could be triggering for someone else. Thus, rather than categorizing content, we focused instead on understanding eating and health cultures online and their effect on our participants' relationship to eating and health.

The broader picture that formed in our fieldwork in both the US and India was a common visual and textual vocabulary shared among our participants that we refer to as “the good life.” By vocabulary, we are referring to not only words, but also images, videos, and discourses—all of which coalesce around broad definitions of wellness, including content that arguably encourages disordered eating tendencies or recycles the same content that was once labeled as pro-ana. While no participants used the phrase “the good life” explicitly, we observed that in both cultures, all of our participants held the belief that pursuing the good life could help them

³We had one disabled participant in our study and their nonbinary experience could have been explored more widely with other participants with a range of disabilities.

achieve “goodness” through healthy eating and exercise. The ideas of “goodness” and “badness” underscored their entire language, beliefs, and cultures around eating and health, and this pursuit of being seen as good in one’s cultural context proliferated through eating, exercise, and lifestyle content. Although in reality being a good person could manifest in many ways (including kindness, altruism etc.), for our participants, the body was a key marker and proxy of how good one is living life. As Frida (24, US) succinctly put it: “In the back of my mind, I thought to be good, you have to be skinny.”

While examinations of online discussions of disordered eating often focus on those which are explicit and pathologized [28, 29], we found participants using words such as “energy,” “mental health,” and “feeling good” to describe their eating and health journeys online, even when recounting restriction and binge cycles. Living the good life meant looking “effortlessly good” and “leading a rich life beyond work.” Participants pursued diets and exercise routines for the sake of “self-improvement,” “mental health,” or “balance,” which were all perceived to be socially acceptable goals, in contrast to wanting to explicitly lose weight and be thin. In the US, the discourse revolved around “mental health,” justifying that taking care of physical health was often done in the service of mental health. US participants sought to enhance their “dateability,” or their ability to find and sustain healthy relationships. In India, the discourse revolved around being “balanced,” and Indian participants were focused on building their “marriageability,” or their ability to attract a suitable husband or wife. However, participants’ social media feeds and practices reinforced the idea that only certain bodies fit the aesthetics of the good life: thin bodies for women, and muscular bodies for men. Many of our participants engaged in harmful disordered eating cycles while pursuing and performing the good life.

It was not enough to live the good life, participants felt compelled to broadcast online that they had attained it. For example, Allie (24, US), a self-described overachiever, medical student, and community builder, experimented with various fad diets. However, she did not call it dieting, instead saying she wanted to be “healthy across the board.” Allie’s Instagram page was carefully curated to project a lifestyle of ease and health: for example, a “candid pose” in an outfit that took many attempts to best highlight the physical results of her diets. In the US, social media was most commonly used by our participants to showcase success in all domains. In India, social media provided participants with access to global norms and practices (e.g., steroid use), filtered through localized influencers, who were also posting similar types of content within this shared vocabulary around the good life.

Seeking to change their bodies was not an irrational exercise: participants were acutely aware that having socially-ideal bodies came with access to real social benefits. For example, Bailey (24, US) was moved up to the front of their dance class after losing weight, and received compliments from peers and the teachers: “I felt attractive and desirable.” Bailey believed that their appearance led them to finally teach classes, and that this newfound privilege provided them an incentive to stay thin. Janet (22, US) was denied entry into a bar because the bouncer judged her body size, while her friends were allowed in. Even when alone, Janet felt penalized;

she enjoyed watching TV dramas but often can’t identify with protagonists because she is physically bigger than the actresses. In contrast to Bailey, Janet was kept to the back in her Chinese folk dance classes, because her teacher said her size was “not aesthetically pleasing.” But as an adult looking back at old photographs of herself, Janet said, “I realized I wasn’t big like she made me think I was!” Abdullah (19, India) said, “Everyone is attracted to the steroid-type body,” where muscles were visually associated with success in career and economic status. In both the US and India, participants felt their bodies were perceived as reflections of their moral choices and society judged them accordingly, which mediated their access to resources and opportunities.

It is worth noting that, despite being steeped in the good life vocabulary online, an emerging element of living a life that was truly fulfilling was having a moderate approach to social media and screen time. Participants felt that living and performing the good life on their phone ultimately didn’t feel like a truly fulfilling life, and all participants sought off-ramps from online worlds into “real life.” Nihal (21, India) shared:

“During the lockdown, I felt no energy and spent so much time on my phone late at night. I had a ‘self-realization’ that I needed to get out there and change my life.”

Yet, the way participants spoke about the binge-purge tendencies around food was similar to how they tried to manage their social media usage. They did not moderate or taper their screen time. Instead, just as they would cut out certain foods for a couple of weeks to lose weight only to then give up the restrictive diet, participants would delete apps only to reinstall them. While they often yearned for alternatives to the online good life, few could resist eventually returning to it. This seeming inescapability of the good life was detrimental to participants because the good life did not challenge messages connected to disordered eating. The good life vocabulary simply obfuscates discussions around disordered eating, disguising this content as productive aspirations to both platforms and users.

4.2 “N=1 Thinking”

Participants overwhelmingly preferred eating and health information from “real” experiences of individuals online over information from institutions or perceived experts. This mirrors previous findings that young people value personal testimony over institutional authority [24]. Many participants thought about eating and health through the lens of identity. In particular, those who identified with marginalized or emergent identities were especially wary of how science has historically harmed and pathologized marginalized people. Within this context, we observed a practice that Krishnendu Ray described as “N=1 thinking,” where people regard personal testimony as valid evidence. Surya (20, India) shared:

“If someone is suggesting something to you and it has worked for them, it gives a little bit more authenticity to it. . . I don’t want experts and their advanced workouts, you don’t know which one is authentic or if it will work for you.”

Participants’ faith in the validity of N=1 thinking was bolstered by the fact that participants were often not watching just one testimonial, but a string of them at a time. Enough repetition of the

same kind of testimony increased their perception that the claims being made were true [30, 31]. For example, Jamuna (23, India) trusted the bhangra dancer diet and exercise tips she heard precisely because they were repeated across many different bhangra influencers.

When shaping their eating and health practices, participants were comfortable favoring personal testimonies over scientific studies because they were not looking to become experts on the topic of health in general, but rather to become experts on their own bodies. Even if a diet or workout suggested by a creator did not work for them, participants did not discount it—they simply assumed it was not right for their bodies, not that the information itself was flawed. Aadita (23, India) described her experience following diets on YouTube and Instagram:

“I didn’t lose 10 kgs in 10 days, I lost 2 kgs, so it just didn’t work for me, but it could work for somebody else.”

The lack of probabilistic thinking around health content in favor of N=1 thinking is problematic because it ensures that online testimony (which could contain harmful advice) cannot be proven false, because it is about experience, not facts. When watching a video about someone else’s new diet, Laney (23, US) shared that she does not worry if that person’s explanation does not sound plausible or stand up to scrutiny. She explained:

“I might not believe the science behind it—your explanation— but I believe your experience is legit. I accept that some things just don’t have an explanation.”

From our participants’ perspective, if institutions are untrustworthy, and anyone’s experience could theoretically be plausible, the only way to know if something works for them is to try it on themselves—which all of our participants did regularly, without vetting information first. Use of N=1 thinking, especially as it relates to eating and health content, means that young people are at risk of experimenting on their own bodies, which may have unintended and lasting consequences on their health and well-being.

4.3 Like-minded, Like-bodied

Participants sought out eating and health information delivered by social media creators who were similar to them; or more specifically, both “like-minded” and “like-bodied.” Like-mindedness is when the user and health-content creator share similar reference points, language, backgrounds, humor, or politics. Like-bodiedness is when the user and health content creator share similar body types, and look to be of the same age and the same race/ethnicity. Like-bodiedness connects people who share seemingly similar genetics, health journeys, and responses to diet and fitness trends. Participants sought out such creators both consciously, by actively searching, and unconsciously, by staying on suggested content in their social media feeds.

Pursuit of the like-minded and like-bodied was one way participants found information that was relevant or personalized, a goal which underscored all of their information-seeking behaviors for health and eating content. Participants were particularly compelled by good life vocabularies produced by like-minded like-bodied individuals, because it was theoretically attainable by someone who

looked and behaved like them. For example, Zuri (26, US) enjoyed Black cottagecore, which represented a modern version of “the little house on the prairie,” because it was “therapeutic” to see aspirational images of Black influencers. When exercising N=1 thinking, participants assigned particular weight to testimonies from the like-minded and like-bodied because they believed that shared similarities raised the chances that a fitness regime would work for them. For example, Janet (22, US) said that because she had a bigger body, she liked following “bigger people who looked like [her],” such as fitness influencer Remy Ashton, who lost weight, gained it back, then lost it again, which mirrored Janet’s own fitness journey.

Participants sought out like-minded, like-bodied messengers to build supportive communities online, often motivated by what they lacked in offline spaces. Many of our participants, particularly those who felt marginalized in the “real world” (namely, participants who were people of color, transgender, and/or disabled) found solace in like-minded, like-bodied people and communities online, where they felt included. For example, Tessa (19, US) found friends and models of friends through the BTS fan community (ARMY) that enabled her to accept herself and overcome her eating disorder:

“I learned through BTS that I am worthy and deserving of love. It was about becoming that for myself and then attracting more people like that in my life. I made women friends because of BTS, through ARMY. I had friends who were women, of all races, of all sexual identities, of all body shapes, and of all walks of life. I could be different and we are all worthy and special.”

But as powerful as it was to see themselves represented online, ultimately our participants all shared an instinct that online communities could not completely substitute for offline engagement, experiences, and connections. Participants were often experiencing the tension between a more appealing online world, where connecting with others felt easier, and the offline world, where people who were like them were not all neatly gathered in one place and where social anxieties abounded.

5 CONCLUSION

Young people have developed a new vocabulary online for discussing eating and health—the good life—that we argue disguises disordered eating conversations. Pursuit of the good life was nominally about achieving holistic health, but required a socially-ideal body to do so, which often led participants to unconsciously adopt disordered eating behaviors while actively eschewing them. Their feeds were filled with like-minded and like-bodied content, which made them feel that the aspirational lifestyle was attainable, and they accepted personal testimonies as trustworthy and valuable (N=1 thinking). Participants ultimately evaluated content through self-experimentation. To support young people and address disordered eating content on social media, we must understand that disordered eating content is often not distinct from mainstream content, and that social media is often a reflection of cultural views on disordered eating and not solely a driver as previous studies have often argued. We need to understand young people’s broader

cultural context, and support them to redirect their search for belonging and purpose in the world into other outlets beyond their bodies to ultimately access a life that is truly good.

ACKNOWLEDGMENTS

We extend our deepest gratitude to Amelia Hassoun, Todd Carmody, and Devika Kumar. We thank them for their many contributions in support of research design, data collection, analysis, and shaping of key concepts in this writing. We would like to thank our experts Hannah Garth, Rebecca Lester, Krishnendu Ray, and Lesley Jo Weaver for sharing their knowledge. We thank Beth Goldberg, Suzannah Isgett, Yasmin Green, and Shira Almeleh for their reviews to improve this manuscript. Finally, we thank our participants for sharing their experiences with us.

REFERENCES

- [1] William A. Haseltine. 2021. How the pandemic is fueling eating disorders in young people (August 2021). *Forbes*. Retrieved from [https://www.forbes.com/sites/williamhaseltine/2021/08/27/how-the-pandemic-is-fueling-eating-disorders-in-young-people/?sh5=\\$2ac426e12226](https://www.forbes.com/sites/williamhaseltine/2021/08/27/how-the-pandemic-is-fueling-eating-disorders-in-young-people/?sh5=$2ac426e12226).
- [2] Yunqi Gao, Nasser Bagheri, and Luis Furuya-Kanamori. 2022. Has the COVID-19 pandemic lockdown worsened eating disorders symptoms among patients with eating disorders? A systematic review. *Journal of Public Health* 30 (2022), 2743–2752. <https://doi.org/10.1007/s10389-022-01704-4>
- [3] Michael P. Levine, Linda Smolak, and Helen Hayden. 1994. The relation of sociocultural factors to eating attitudes and behaviors among middle school girls. *The Journal of Early Adolescence* 14, 4 (1994), 471–490. <https://doi.org/10.1177/0272431694014004004>
- [4] Julia Hannon, Lindsey Eunseon, and Calum Munro. 2017. The patient experience of illness, treatment, and change, during intensive community treatment for severe anorexia nervosa. *Eating Disorders* 25, 4 (April 2017), 279–296. <https://doi.org/10.1080/10640266.2017.1318626>
- [5] Michael Strober and Craig Johnson. 2012. The need for complex ideas in anorexia nervosa: Why biology, environment, and psyche all matter, why therapists make mistakes, and why clinical benchmarks are needed for managing weight correction. *International Journal of Eating Disorders* 45, 2 (January 2012), 155–178. <https://doi.org/10.1002/eat.22005>
- [6] Kristen M. Culbert, Sarah E. Racine, and Kelly L. Klump. 2015. Research Review: What we have learned about the causes of eating disorders—a synthesis of sociocultural, psychological, and biological research. *Journal of Child Psychology and Psychiatry* 56, 11 (2015), 1141–1164. <https://doi.org/10.1111/jcpp.12441>
- [7] Eileen P. Anderson-Fye. 2018. Cultural influences on body image and eating disorders. *The Oxford Handbook of Eating Disorders* 1 (2018), 187–208. Oxford University Press, New York, USA.
- [8] Rebecca Lester. 2021. Q&A with Anthropologist Rebecca Lester: What We Get Wrong about Eating Disorders (November 2020). UC Press. Retrieved from <https://www.ucpress.edu/blog/53148>.
- [9] Jordan Bissell. 2018. Vegan YouTube Stars are Held to Impossible Standards. *The Atlantic*. Retrieved from <https://www.theatlantic.com/technology/archive/2018/05/vegan-youtube-stars-are-held-to-impossible-standards/560807>.
- [10] James Hamblin. 2018. The Jordan Peterson All-Meat Diet. *The Atlantic*. Retrieved from <https://www.theatlantic.com/health/archive/2018/08/the-peterson-family-meat-cleanse/567613>.
- [11] Linda Charmaraman. 2021. Social media gives support to LGBTQ youth when in-person communities are lacking. *The Conversation*. Retrieved from <https://theconversation.com/social-media-gives-support-to-lgbtq-youth-when-in-person-communities-are-lacking-16625>.
- [12] Olu Jenzen. 2017. Trans Youth and Social Media: Moving Between Counterpublics and the Wider Web. *Gender, Place, and Culture: A Journal of Feminist Geography* 24, 11 (November 2017), 1626–1641. <https://doi.org/10.1080/0966369X.2017.1396204>.
- [13] David Buckingham (Ed.). 2008. *Introduction to Identity*. Youth, Identity, and Digital Media. MIT Press.
- [14] Amanda Lenhart and Kellie Owens. 2021. *The Unseen Teen: The Challenges of Building Healthy Tech for Young People*. Data and Society. Retrieved from <https://datasociety.net/library/the-unseen-teen/>.
- [15] Mary Madden. 2021. Gen Z refuses to be locked in. *New Public Magazine*. Retrieved from <https://newpublic.org/article/1654/gen-z-refuses-to-be-locked-in>.
- [16] Alice E. Marwick and Danah Boyd. 2011. I tweet honestly, I tweet passionately: Twitter users, context collapse, and the imagined audience. *New Media and Society* 13, 1 (February 2011), 114–133. <https://doi.org/10.1177/14614448103653>
- [17] Zoya Unni and Emily Weinstein. 2021. Shelter in Place, Connect Online: Trending TikTok Content During the Early Days of the U.S. COVID-19 Pandemic. *Journal of Adolescent Health* 68, 5 (May 2021), 863–868. <https://doi.org/10.1016/j.jadohealth.2021.02.012>
- [18] Guy Cafri, Yuko Yamamiya, Michael Brannick, and J. Kevin Thompson. 2005. The influence of sociocultural factors on body image: a meta-analysis. *Clinical Psychology: Science and Practice* 12, 4 (2005), 421–433. <https://doi.org/10.1093/clipsy.bpi053>
- [19] Stephen P. Lewis and Alexis E. Arbutnot. 2012. Searching for thinspiration: the nature of internet searches for pro-eating disorder websites. *Cyberpsychology, Behavior, and Social Networking* 15, 4 (April 2012), 200–204. <https://doi.org/10.1089/cyber.2011.0453>
- [20] Bryn Austin. 2021. How social media's toxic content sends teens into 'a dangerous spiral'. Harvard T.H. Chan School of Public Health. Retrieved from <https://www.hsph.harvard.edu/news/features/how-social-medias-toxic-content-sends-teens-into-a-dangerous-spiral>.
- [21] Rebecca Greenfield. 2012. To Ban or Not to Ban: How Do You Solve the Problem of Thinspo?. *The Atlantic*. Retrieved from <https://www.theatlantic.com/technology/archive/2012/08/ban-or-not-ban-how-do-you-solve-problem-thinspo/32471/>.
- [22] Jeannie Suk Gersen. 2021. What If Trigger Warnings Don't Work?. *The New Yorker*. Retrieved from <https://www.newyorker.com/news/our-columnists/what-if-trigger-warnings-dont-work>.
- [23] Amanda Mull. 2019. Putting Kids on Diets Won't Solve Anything. *The Atlantic*. Retrieved from <https://www.theatlantic.com/health/archive/2019/08/weight-watchers-diet-app-kids/596422/>.
- [24] Amelia Hassoun, Ian Beacock, Sunny Consolvo, Beth Goldberg, Patrick Gage Kelley, and Daniel M. Russell. 2023. Practicing Information Sensibility: How Gen Z Engages with Online Information. In 99). In *Proceedings of the 2023 CHI Conference on Human Factors in Computing Systems*. ACM, Hamburg, Germany, 1–17. <https://doi.org/10.1145/3544548.3581328>
- [25] Kathy Charmaz. 2006. *Constructing grounded theory: A practical guide through qualitative analysis*. Sage.
- [26] Jane Mills, Ann Bonner, and Karen Francis. 2005. The development of constructivist grounded theory. *International Journal of Qualitative Methods* 5, 1 (2005), 25–35. <https://doi.org/10.1111/j.1440-172X.2006.00543.x>
- [27] Johnny Saldana. 2021. *The coding manual for qualitative researchers*. Sage.
- [28] Susan Bordo. 1993. *Unbearable Weight : Feminism, Western Culture, and the Body*. University of California Press.
- [29] Naomi Wolf. 1990. *The Beauty Myth*. Chatto and Windus, London, UK.
- [30] Aumyo Hassan and Sarah J. Barber. 2021. The effects of repetition frequency on the illusory truth effect. *Cognitive Research: Principles and Implications* 6 (May 2021), 1–12.
- [31] Lisa K. Fabio, Nadia M. Brashier, B. Keith Payne, and Elizabeth J. Marsh. 2015. Knowledge does not protect against illusory truth. *Journal of Experimental Psychology: General* 144, 5 (2015), 993–1002. <https://doi.org/10.1037/xge0000098>

A APPENDICES

A A.1 RECRUITMENT DEMOGRAPHICS (APPENDIXA1)

Table 1: Demographics of sample in United States and India

US		INDIA	
GEOGRAPHY		GEOGRAPHY	
Urban	11	Urban	15
Suburban	5	Semi-urban	3
Rural	5	Semi-rural	3
GENDER		GENDER	
Female	10	Female	11
Male	5	Male	10
Trans	4		
Nonbinary	2		
RACE		RELIGION	
Asian	2	Hindu	14
Black	4	Muslim	2
Latino	4	Christian	2
White	11	Sikh	2
		Jain	1